NCCP 00587 · VERSION 2

# Your treatment, explained

**Relapsed small-cell lung cancer** · NCCP 00587 v2 · Document revised 4 November 2026

**Your personal plan:** this is the standard regimen schedule. Your team confirms your prescribed doses, cycles, treatment changes and appointments in your written plan.

Topotecan capsules · small-cell lung cancer

21-day cycle

Source reconciled; independent clinical, pharmacy and patient/PPI review pending

ACT ON SYMPTOMS, NOT THE CALENDAR

## When to get help

### Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

### Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Call oncology immediately for fever or suddenly feeling unwell, unusual bleeding, severe tummy pain, persistent vomiting or diarrhoea, or new cough or breathlessness. Do not wait for the next cycle. Contact the team immediately for more than 3 episodes of diarrhoea in a day, or sooner if you feel unwell.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

Open your CUH contact / treatment card

Sources: nccp · smpc

**Choose a topic to read**

YOUR COMPLETE REFERENCE

# Read what you need, when you need it.

Start with the overview, explore the cycle, or go straight to a question. Urgent advice is always available.

- How each drug works
- Your treatment course
- Your 21-day timeline
- Baseline and cycle-specific checks
- Whole treatment, scans and checks
- Tablets, visits and practical planning
- My treatment
- My schedule
- Between treatments
- When to call
- Tests and review
- Supportive medicines
- Understanding the evidence
- Questions to ask
- Sources

UNDERSTAND YOUR TREATMENT

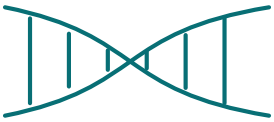
# Your chemotherapy

This treatment aims to slow or shrink small-cell lung cancer that has returned after treatment, when repeating the first treatment is not appropriate. Benefit varies; your team explains the aim for you.

TOPOISOMERASE-I INHIBITOR CHEMOTHERAPY

## Topotecan capsules

Hycamtin; supplied brand may vary



### Disrupts DNA copying and repair

Topotecan blocks topoisomerase I, an enzyme that relieves strain while DNA is copied. Unrepaired DNA breaks can stop dividing cancer cells; healthy blood-forming and gut cells can also be affected.

Medicine reaches cells → DNA damage persists → Cell division disrupted

### What to watch for

Call oncology immediately for fever or suddenly feeling unwell, unusual bleeding, severe tummy pain, persistent vomiting or diarrhoea, or new cough or breathlessness. Do not wait for the next cycle. Contact the team immediately for more than 3 episodes of diarrhoea in a day, or sooner if you feel unwell.

Sources: nccp · smpc

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

**Explore your existing anatomy and side-effect map**

THE BIGGER TREATMENT PLAN

# Your treatment course

UP TO 6 CYCLES

### Chemotherapy course

1 2 3 4 5 6

Take the prescribed topotecan capsules once daily on Days 1–5 of a 21-day cycle. Days 6–21 have no planned topotecan. Up to 6 cycles are planned, with earlier changes if needed.

INDIVIDUAL PLAN

### Review and next plan

**Review**

Your team reviews benefit and side effects and agrees whether to continue, change or stop.

Relative educational plan; your team confirms actual appointments and changes.

Sources: nccp

BASELINE → TREATMENT → FOLLOW-UP

## My tests and checks

| Check | Before treatment | During the cycle | Before next course | After treatment |
|---|---|---|---|---|
| **Safety blood tests** | Blood count, kidney and liver profile | Report symptoms; extra tests if needed | Blood count, kidney and liver review | Follow abnormal results as advised |
| **Disease assessment** | Confirm treatment aim | Symptoms reviewed | Scans or other tests on consultant plan | Agree next treatment or follow-up |

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00587 v2 · official regimen

THE WHOLE TREATMENT JOURNEY

# Topotecan capsules · small-cell lung cancer: treatment and checks

## Relapsed small-cell lung cancer

Take the prescribed topotecan capsules once daily on Days 1–5 of a 21-day cycle. Days 6–21 have no planned topotecan. Up to 6 cycles are planned, with earlier changes if needed.

**Read from left to right.** Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

On a small screen, swipe the map sideways. The checks below give the same timing in words.

**Treatment, tests and reviews across the course**

| Care / check | Before treatment | Treatment days | Between courses | Course review |
|---|---|---|---|---|
| **Treatment** NCCP 00587 | Confirm capsule strength, number, dosing days, storage and contact numbers with the pharmacist. | Take the prescribed topotecan capsules once daily on Days 1–5 of a 21-day cycle. Days 6–21 have no planned topotecan. Up to 6 cycles are planned, with earlier changes if needed. | Follow support plan and report symptoms | Review benefit/toxicity; next plan |
| **Safety tests** NCCP 00587 | Blood count, kidney/liver tests | Review symptoms; extra tests if needed | Blood count, kidney and liver tests before treatment and before each cycle. Scans and follow-up follow the consultant plan. | Follow abnormal results |

## What each check is for

1

**Before each course**

Before starting

Safety and plan may change

**Ask / act:** Team confirms next treatment

Sources: NCCP 00587

## At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-02 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.29

YOUR CYCLE, DAY BY DAY

# Your 21-day cycle.

Select a day to explore what to watch for. These are possible patterns, not a promise about when symptoms start or stop.

> The full information is available below. Interactive day, phase and date controls require JavaScript; use “Read the whole cycle as text” for the complete timeline.

Your date stays on this page only. It is not saved or added to a QR code.

**Medicines on Day 1:** Topotecan capsules

On a small screen, swipe the chart sideways. Each day is a keyboard-accessible button.



| | | |
|---|---|---|
| Prescribed capsules | IV | |
| No planned chemotherapy | | Blood-count and symptom recovery varies. |

Effects vary; no risk curve or guaranteed recovery day is shown.

DAY 1

### Treatment days

Take the prescribed topotecan capsules once daily on Days 1–5 of a 21-day cycle. Days 6–21 have no planned topotecan. Up to 6 cycles are planned, with earlier changes if needed.

**Your next step:** Follow the confirmed prescription; report problems promptly.

The next cycle starts 21 days after this one, if your team confirms it.

> Do not use this page to calculate or change doses.

**Read the whole cycle as text**

### Day 1–5: Treatment days

Take the prescribed topotecan capsules once daily on Days 1–5 of a 21-day cycle. Days 6–21 have no planned topotecan. Up to 6 cycles are planned, with earlier changes if needed. Follow the confirmed prescription; report problems promptly.

### Day 6–21: Between treatments

No planned chemotherapy; continue prescribed support and monitoring. Do not restart until the team confirms the next cycle.

Sources: nccp

PRACTICAL TREATMENT GUIDE

## Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

### Before treatment

Confirm capsule strength, number, dosing days, storage and contact numbers with the pharmacist.

STEP 2

### Treatment visit or home course

Swallow capsules whole with water; do not open, chew or crush them. Topotecan can be taken with or without food. Keep it refrigerated at 2–8°C; do not freeze. The pharmacy label gives the exact strengths and number of capsules.

STEP 3

### Bring to review

Bring your medicine list, dose record and questions; report new symptoms before starting another course.

Open practical treatment guide · Printable practical guide

Sources: NCCP 00587 v2 · official regimen. Source reconciled; independent clinical, pharmacy and patient/PPI review pending.

MAKE THE PLAN PRACTICAL

# Support around the medicines

### 1 Sickness and bowel symptoms

Use your prescribed prevention or rescue medicine. Report persistent vomiting, diarrhoea or inability to drink; fever with severe tummy pain needs urgent assessment.

### 2 Blood tests and review

Blood count, kidney and liver tests are checked before treatment and before each cycle. Extra tests depend on symptoms and results.

### 3 Pregnancy, fertility and medicines

Discuss fertility and contraception before treatment. Tell the team about pregnancy, breastfeeding, all medicines, supplements and vaccines. Check before changing any medicine.

### 4 Missed or vomited capsules

Do not double a missed dose or take replacement capsules after vomiting without team advice. Record what happened, contact the oncology pharmacist/team, and follow the next scheduled dose in the agreed plan.

### 5 Safe handling

Swallow capsules whole with water; do not open, chew or crush them. Topotecan can be taken with or without food. Keep it refrigerated at 2–8°C; do not freeze. The pharmacy label gives the exact strengths and number of capsules. Keep away from children; ask the pharmacy how to return unused or damaged capsules.

Sources: nccp · smpc · ncis

TREATMENT REFERENCE · 01

# My treatment

This treatment aims to slow or shrink small-cell lung cancer that has returned after treatment, when repeating the first treatment is not appropriate. Benefit varies; your team explains the aim for you.

### How it works

Topotecan blocks topoisomerase I, an enzyme that relieves strain while DNA is copied. Unrepaired DNA breaks can stop dividing cancer cells; healthy blood-forming and gut cells can also be affected.

Sources: nccp · smpc

### Evidence and limits

An older randomized trial found longer survival and slower loss of quality of life when oral topotecan was added to supportive care in patients unsuitable for standard intravenous treatment. These historical groups do not predict an individual outcome or compare every treatment available today. Treatment choice also depends on previous response, time since treatment, fitness and preferences.

Sources: trial

TREATMENT REFERENCE · 02

# My schedule

Take the prescribed topotecan capsules once daily on Days 1–5 of a 21-day cycle. Days 6–21 have no planned topotecan. Up to 6 cycles are planned, with earlier changes if needed.

**Administration**

Swallow capsules whole with water; do not open, chew or crush them. Topotecan can be taken with or without food. Keep it refrigerated at 2–8°C; do not freeze. The pharmacy label gives the exact strengths and number of capsules.

Sources: nccp · smpc

TREATMENT REFERENCE · 03

# Between treatments

Keep your prescription and contact plan to hand.

**Sickness and bowel symptoms**

Use your prescribed prevention or rescue medicine. Report persistent vomiting, diarrhoea or inability to drink; fever with severe tummy pain needs urgent assessment.

Sources: nccp · smpc

**Blood tests and review**

Blood count, kidney and liver tests are checked before treatment and before each cycle. Extra tests depend on symptoms and results.

Sources: nccp · smpc

**Pregnancy, fertility and medicines**

Discuss fertility and contraception before treatment. Tell the team about pregnancy, breastfeeding, all medicines, supplements and vaccines. Check before changing any medicine.

Sources: nccp · smpc

**Missed or vomited capsules**

Do not double a missed dose or take replacement capsules after vomiting without team advice. Record what happened, contact the oncology pharmacist/team, and follow the next scheduled dose in the agreed plan.

Sources: nccp · smpc

**Safe handling**

Swallow capsules whole with water; do not open, chew or crush them. Topotecan can be taken with or without food. Keep it refrigerated at 2–8°C; do not freeze. The pharmacy label gives the exact strengths and number of capsules. Keep away from children; ask the pharmacy how to return unused or damaged capsules.

Sources: nccp · smpc

TREATMENT REFERENCE · 04

# When to call

Call oncology immediately for fever or suddenly feeling unwell, unusual bleeding, severe tummy pain, persistent vomiting or diarrhoea, or new cough or breathlessness. Do not wait for the next cycle. Contact the team immediately for more than 3 episodes of diarrhoea in a day, or sooner if you feel unwell.

**Blood counts**

An infection can be serious even after treatment days have finished. A normal earlier blood test does not rule out a new problem.

Sources: nccp · smpc

TREATMENT REFERENCE · 05

## Tests and review

Blood count, kidney and liver tests before treatment and before each cycle. Scans and follow-up follow the consultant plan.

**Scans and tumour markers**

Your consultant decides which are relevant and when. Do not assume a fixed imaging interval or that tumour markers are required for everyone.

Sources: nccp · smpc

**After treatment**

Agree follow-up, symptom support and the next plan before the course ends.

Sources: nccp · smpc

## Why this treatment may be offered

An older randomized trial found longer survival and slower loss of quality of life when oral topotecan was added to supportive care in patients unsuitable for standard intravenous treatment.

These historical groups do not predict an individual outcome or compare every treatment available today. Treatment choice also depends on previous response, time since treatment, fitness and preferences.

Sources: trial · nccp

PREPARE FOR THE CONVERSATION

# Questions worth asking

**Why can treatment be delayed?**

Blood results or symptoms may make a delay or dose change safer. Your team confirms when to restart.

**Are capsules gentler than an infusion?**

No. Oral chemotherapy can cause serious toxicity and needs blood tests and review. Oral and intravenous doses cannot be exchanged.

**When will I have scans?**

Your consultant sets the timing for your disease and treatment plan. No universal scan or tumour-marker interval is specified here.

> **Try explaining the plan back:** Explain which days you take treatment, when your next blood tests are due and whom you will call tonight if unwell.

SOURCE TRANSPARENCY

# Where this information comes from

Regimen schedule: NCCP 00587 v2. Medicine explanations and safety information: sources below. The timeline is an educational illustration, not a measured risk curve or an individual prediction.


- NCCP 00587 v2 · official regimen
- EMA Hycamtin product information
- NCCP NCIS medical oncology antiemetic guidance V8
- O’Brien 2006 oral topotecan plus supportive care

SACTCheck · Developed by Dr Paul O'Brien · Sources checked 2026-11-02.
This information supports your treatment discussion. Your oncology team confirms your personal treatment and urgent-care plan.

## Written source addresses

https://healthservice.hse.ie/documents/6886/587_V2_Topotecan_PO.pdf

https://www.ema.europa.eu/en/documents/product-information/hycamtin-epar-product-information_en.pdf

https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf

https://pubmed.ncbi.nlm.nih.gov/17135646/